

Code Status, Do-Not-Resuscitate, and Advance Directives

Policy Number	POL-PC-063	Owner / Department	Medical Staff / Nursing
Effective Date	2023-02-15	Version	2.1
Last Revised	2026-01-18	Next Review	2028-01-18
Approved By	Ethics Committee	Applies To	All inpatients and observation patients; all licensed providers and nursing staff

1. Purpose

To honor patients' rights to make decisions about resuscitation and life-sustaining treatment, to document code status clearly, and to recognize advance directives, consistent with the Patient Self-Determination Act and CMS Conditions of Participation (42 CFR 482.13).

2. Scope

Applies to all patients. On admission, every patient is asked whether they have an advance directive and is informed of their right to make one; existing directives are obtained and placed in the record.

3. Definitions

- **Advance directive** — a written instruction, such as a living will or a durable power of attorney for health care, recognized under state law, stating a person's wishes or naming an agent.
- **Healthcare agent / proxy** — the person authorized to make medical decisions when the patient lacks capacity.
- **POLST** — Physician/Provider Orders for Life-Sustaining Treatment: portable medical orders for seriously ill patients.
- **Decision-making capacity** — the ability to understand, appreciate, reason about, and communicate a choice regarding a specific decision.

4. Code Status Definitions

Code status is a provider order based on a documented goals-of-care conversation with the patient or their surrogate. In the absence of a valid order or directive, the default is Full Code.

Code status	Meaning
Full Code	All resuscitative measures, including chest compressions, defibrillation, intubation, and advanced cardiac life support.
DNR / DNAR	Do Not Resuscitate / Attempt Resuscitation: no CPR or ACLS in the event of cardiac or respiratory arrest; all other indicated treatment continues.
DNI	Do Not Intubate: no endotracheal intubation or mechanical ventilation; other measures per the order.

Code status	Meaning
Limited / partial	Specific interventions elected or declined per a documented order (e.g., defibrillation yes, intubation no).

5. Advance Directive Levels

Instrument	What it does
Living will	States the patient's wishes about life-sustaining treatment if they become unable to decide.
Durable power of attorney for health care	Names a healthcare agent to make decisions when the patient lacks capacity.
POLST	Portable provider orders that travel with a seriously ill patient across settings; actionable immediately.

6. Honoring and Changing Code Status

IMPORTANT — A DNR/DNAR order must be a current, documented provider order in the medical record before resuscitation is withheld. In the absence of a valid DNR order or an actionable directive, staff must initiate full resuscitation. A patient with capacity may change their code status at any time; update the order immediately and communicate it to the care team.

- Confirm the code-status order at every handoff and transfer of care.
- If a patient lacks capacity and no directive or agent exists, follow the state surrogate hierarchy and involve the attending; consult the Ethics Committee for conflict or uncertainty.
- Reconcile any conflict between a directive, a POLST, and the current order before acting, favoring the patient's known current wishes.

7. Severity & Reporting

Events are classified and reported under POL-RM-013. Resuscitation performed contrary to a valid DNR order, or resuscitation withheld without a valid order, is a SEV-1 event — notify the attending provider, Charge Nurse, Nursing Supervisor, Risk Management, and the Administrator-on-Call immediately and within 1 hour. A documentation discrepancy caught before it affects care is a near miss, reported within 24 hours. Any outcome resulting from a code-status error is disclosed to the patient/family under POL-PC-067.

8. References

- Patient Self-Determination Act (42 USC 1395cc(f)).
- CMS Conditions of Participation, 42 CFR 482.13 — Patients' Rights.
- The Joint Commission, Provision of Care and Rights-and-Responsibilities standards.
- Related: POL-PC-067 (disclosure of unanticipated outcomes), POL-RM-013 (severity & reporting).